



PROCEDURE

Provision of Further Opinions under the Mental Health Act 2014

Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	All NMHS MH Areas

1. Aim

All involuntary patients (inpatients, community treatment order patients and mentally impaired accused patients), their families, carers and nominated person will be informed of their right to request a further opinion, both verbally and in writing, during their contact with the mental health services (inpatient and community). This is irrespective of whether or not the person has been reviewed by another psychiatrist during that admission.

2. Background

The [Mental Health Act 2014](#) (MHA 2014) recognises that further opinions are an important patient right, and safeguards this right by stating (s182 MHA 2014) that when either a patient, patient's carer, close family member or nominated person is dissatisfied with the clinical diagnosis or treatment provided, they may request a further opinion regarding whether the treatment is appropriate and the patient's psychiatrist must organise the further opinion as soon as practicable, unless the request is from a personal support person and the patient objects. [The Chief Psychiatrist's Guidelines](#) under section 547(1)(c) will aim to ensure the independence of psychiatrists from whom further opinions are obtained.

The person requesting the further opinion should specify the reason for the further opinion and what outcome they are looking for. This may be made 'orally or in writing' and is to be documented on the **Request For A Further Opinion Template (Appendix 1)**.

If a patient wishes to have their involuntary status reviewed (inpatient or community involuntary status), they should be directed to the [Mental Health Review Tribunal](#).

3. Risk

Should further opinions not be provided in a timely manner, the relevant service is likely to receive complaints via the [Office of the Chief Psychiatrist](#) (OCP) and the Government of Western Australia [Health and Disability Services Complaints Office](#) (HADSCO).

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐

Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Process

A further opinion must be independent of the treating team. The patient must be provided with a range of options by the treating psychiatrist (or his/her delegate) in relation to the provision of further opinions.

1. When a person requests the further opinion, the treating psychiatrist or delegate will present them with a range of options for how that further opinion is to be sought. These options are:
 - a. A further opinion from a psychiatrist within the same health service site.
 - b. A further opinion from a psychiatrist from a different site but within the same health service.
 - c. A further opinion from a psychiatrist from a different health service.
 - d. A further opinion from a private psychiatrist.

The option of using a private psychiatrist will first be subject to a risk assessment as to whether the patient is able to leave the hospital. If this is not possible then it may not be viable for the patient to attend an examination by a private psychiatrist. The private psychiatrist may visit the hospital to see the patient however this will increase the cost of the further opinion.

When presented with the options, the patient will be informed of the competing considerations – for instance the patient may find a further opinion from a psychiatrist from the same site or same service more convenient and the provision of the further opinion is likely to be completed in a more timely fashion. A further opinion from a private psychiatrist will incur a cost to the patient requesting the further opinion.

The treating psychiatrist or delegate will then notify the Head of Clinical Service (HOCS) of the request for the further opinion, noting the option chosen by the patient.

2. If the patient's preferred option is a further opinion from a private psychiatrist then the patient requesting the further opinion can instruct any suitably qualified Psychiatrist. [RANZCP provide a search engine for the purpose of finding a relevant practitioner.](#)
3. If the patient's preferred option is a psychiatrist from another site or health service, the HOCS or delegate will contact a HOCS of another site/service and organise for an available psychiatrist to provide the further opinion. If the patient is from a specialised group (e.g. CAMHS or older adult) this is to be taken into account as far as possible when identifying an appropriate psychiatrist.

The Further Opinion request will be documented on the Further Opinion Template and filed in the clinical notes on completion. Processes exist at each service to collate these. The advantages and disadvantages of the options listed above (timeliness, costs and degree of independence) should be explained to the patient/ carer/ nominated or support person so that they may make as informed a decision as possible.

With travel, time for an assessment interview, documentation and ensuring a letter is written with a copy for the patient, a minimum of 4 to 6 hours of a psychiatrist's time will be required.

For public psychiatrists to progress this without disrupting their clinical demands and impacting unreasonably on other consumers, the NMHS MH Medical Advisory Committee estimates a wait of 2 weeks may not be uncommon.

Arrangements for the provision of the opinion should be instigated ‘as soon as practicable’.

The Treating Team will document in the Integrated Progress Notes:

- When the request has been received,
- When the opinion has been provided
- Utilise the Further Opinion Template (**Appendix 1**)

The patient’s Psychiatrist must, as soon as practicable after obtaining the further opinion:

- File the opinion and give a copy to the patient.
- If the opinion was requested by a person other than the patient – give a copy to that other person’.

Patients may request an additional opinion (Sec 183 MHA 2014), however this may be refused if the treating psychiatrist in consultation with the HOCS/ Medical Co Director/ DCS or Chief Psychiatrist are satisfied it is not warranted (Sec 183 (2)). If the additional opinion is refused by the patient’s Psychiatrist, they must provide a copy of the reasons for the decision to the patient, other requestor and the Chief Psychiatrist. The Chief Psychiatrist will then ‘confirm or refuse to confirm’ this decision.

5. Compliance, monitoring and evaluation

The treating psychiatrist is responsible for flagging to HOCS any problems relating to timescales not being met. The HOCS is responsible for delegating the task of recording and documenting when requested opinions have been completed, in order to monitor timeliness.

Related internal policies, procedures and guidelines
Mental Health Act 2014

References
The Chief Psychiatrist’s Guidelines Mental Health Review Tribunal Office of the Chief Psychiatrist Health and Disability Services Complaints Office

Policy Sponsor	Director of Clinical Services MHPHDS				
Policy Contact	Director of Clinical Services MHPHDS				
Date First Issued:	14/06/2013	Last Reviewed:	20/06/2022	Review Date:	20/06/2026
Approved by:	Director of Clinical Services MHPHDS			Date:	20/06/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	20/06/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management ☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers,				
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Document Version Control			
Date	Version	Author	Notes
14/06/2013	1.0	MHEG	Initial Endorsement
07/06/2018	1.3	A/Policy Audit Officer	Review date amended to reflect endorsement 30/11/2015. All hyperlinks repaired.
02/04/2019	2.0	Consultant Psychiatrist Mirrabooka MH	Scheduled review, no major change to the content of this document, updated template, hyperlinks, changed policy document type to procedure, change in policy sponsor.

20/06/2022	3.0	Director of Clinical Services MHPHDS	Scheduled review, no major changes to the content of this document, updated template and hyperlinks
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Appendix 1: Department of Health Request For A Further Opinion

 Department of Health NB: THIS IS NOT A LEGAL FORM	Please use ID label or block print	
	SURNAME	UMRN
	GIVEN NAMES	CMBI
	BIRTHDATE	SEX
	ADDRESS	
REQUEST FOR A FURTHER OPINION		
REQUEST: Date of request for Further Opinion: Date patient seen for Further Opinion: Place where examination occurred: This Further Opinion pursuant to s.182 (2) of the <i>Mental Health Act 2014</i> has been requested by: Patient ☐ Name Personal Support Person ☐ (Family/Carer) Nominated Person ☐ Chief Psychiatrist ☐ Mental Health Advocacy Service ☐ Treating Psychiatrist: Service: Desired outcome of Further Opinion (from person making request): ☐ Review of Diagnosis ☐ Review of Treatment (including medication) ☐ Review of continuation order for CTO via further opinion. (NB - Reviews for MHA status are to be referred to Mental Health Tribunal.) Reason/s for requesting a further opinion (As at time of examination): Advanced Health Directive (tick applicable): Yes ☐ No ☐ Unknown ☐ Interpreter required: Yes ☐ No ☐ If Yes, specify language Interviews (tick all applicable): Patient ☐ Personal Support Person ☐ Treating Psychiatrist ☐ Other (describe) Other source of information: Limitations to available information: Patient is seeking a further opinion from: ☐ A psychiatrist within the same health service site ☐ A psychiatrist from a different site but within the same health service ☐ A psychiatrist from a different health service ☐ A private Psychiatrist		

FURTHER OPINION REQUEST